National Cancer Control Programme (NCCP) 00794 · clinician draft

Sacituzumab govitecan (Trodelvy)

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · Sacituzumab govitecan (Trodelvy)

Active medicines: Sacituzumab govitecan (Trodelvy)

Emetogenic risk: High · sacituzumab govitecan

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1 before treatment
Ondansetron	16 mg orally	Day 1 before treatment Review QT risk, hepatic impairment, constipation and interacting medicines.
Dexamethasone	12 mg orally	Day 1 before treatment Reconcile all steroid premedication; do not duplicate.
Olanzapine	5 mg orally once daily	Days 1–4 Review sedation, falls, glucose and QT risk. Avoid routine combination with metoclopramide because of additive extrapyramidal effects; select rescue individually.
Aprepitant	80 mg orally once daily	Days 2–3
Dexamethasone	8 mg orally once daily	Days 2–4 Confirm suitability and total steroid exposure.
Metoclopramide	10 mg orally up to three times daily	Rescue use / duration to be completed by prescriber Review renal function and neurological contraindications; avoid automatic prolonged use.

Additional phase-specific support

- This is a high-emetic-risk regimen. Preventive antiemetics are needed for both Day 1 and Day 8, with take-home medicines and clear instructions. The steroid used as an infusion premedicine also provides antiemetic cover, so the team must reconcile the total dose.
- Report diarrhoea early. The team assesses infection, dehydration and electrolyte loss, and supplies an appropriate antidiarrhoeal prescription. Follow the written oncology instructions; do not improvise repeated high doses of loperamide.
- granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) injections may be considered from cycle 1 if the risk of febrile neutropenia is increased, or after significant neutropenia. Product and timing depend on the two-infusion schedule and the prescribing plan.
- The unit gives premedicines to reduce reaction risk and observes you during and after each infusion. Tell staff immediately if you feel different or unwell.
- This can be an acute cholinergic effect and needs staff assessment. Atropine may be used for later infusions when appropriate; tell the team if you have glaucoma.
- This is the high-risk national antiemetic template anchored to this infusion day. Day8 repeats prophylaxis only if treatment proceeds; do not print both scripts as simultaneous orders. Antihistamine/antipyretic infusion premedication and G-CSF are separate regimen-specific decisions.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00794 v3 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-25 · SACTCheck v0.84.21 · Regimen-level drafts; prescriber and oncology-pharmacy verification required